



CHIROPRACTIC
SLEEP ACADEMY

CHIROPRACTIC SLEEP ACADEMY®

The Workbook

*Bridging Biomechanics, Airway Science, and Sleep Medicine for the
Doctor of Chiropractic — Credible Edition*

Dr. Robert Zeravica, DC

Founder, Chiropractic Sleep Academy® · Developer, VAET™

www.ChiropracticSleepAcademy.net

CE approved by the California Board of Chiropractic Examiners

Version 2 · organized as a 4-week program

FRONT MATTER

Disclaimers & Foundation

Education for Doctors of Chiropractic. Does not diagnose, treat, or prescribe for any disease; not individualized medical advice. Sleep studies are **physician-read**; the DC's role is structural evaluation, education, coordination, and — within a state's scope — clinical interpretation. Moderate-to-severe disease is **co-managed**. Nothing here replaces CPAP, oral appliances, surgery, or medication, and no patient is advised to stop prescribed therapy.

FOUNDATION — FOUNDER'S FAITH BASIS (OPTIONAL TO THE SCIENCE)

Two registers. The clinical science stands on its own — no belief required. This states the founder's personal *why*; it is not part of the clinical argument.

Secular statement: The body is a living, whole system worth caring for with rigor and respect.

Founder's faith foundation: the body as a living temple (1 Cor. 6:19-20; Rom. 12:1; Gen. 2:7) — caring for structure is stewardship.

Mission

Equip DCs to **raise awareness, screen, coordinate physician-read testing, educate, plan structural care, co-manage, and re-evaluate** patients with airway-related sleep concerns — restoring structure so sleep, breathing, and function can improve, always measured and within scope.

About the Author

Dr. Robert Zeravica, DC — Founder, Chiropractic Sleep Academy® · Developer, Viscerocranial Airway Expansion Therapy (VAET™).

- Doctor of Chiropractic — Los Angeles College of Chiropractic (1999); active California license
- B.S., Business Administration — Pepperdine University
- 100-hour certification in **Applied Kinesiology** (ICAK)
- Certified in **Cranial Facial Release (CFR)** and **Bilateral Nasal Specific (BNS)** (balloon-instrument)
- Certified in **Manipulation Under Anesthesia (MUA)**
- Certified in **Balancing Body Chemistries** (clinical nutritional supplementation)
- **Sacro-Occipital Technique (SOT)** — in progress
- **U.S. Marine Corps** NCO — 4th Force Reconnaissance & 3rd ANGLICO (Air Naval Gunfire Liaison Company)

 **Confirm official titles/abbreviations before print.** Verify each certification's exact name and abbreviation with its certifying body. The Applied Kinesiology credential is the **100-hour ICAK certification** — *not* the DIBAK diplomate. Spelled-out certifications are preferred over acronym strings.

PART I

Foundations of Structural Airway Chiropractic

Course map: Week 1 — Sleep Knowledge

Ch 1 — History & Philosophy

Chiropractic on the principle that **structure governs function** (D.D. Palmer; A.T. Still founded osteopathy contemporaneously). CSA is **chiropractic** — studies the whole body, learns from any tradition, not "osteopathic."

Ch 2 — From Dogma to Discernment GOVERNING METHOD

We do not judge truth claims; we judge clinical consequence. Keep what works, report honestly (including non-responders). Every model labeled **established** or **hypothesis under investigation**.

Ch 3 — Innate Intelligence & PRI/PRM

Innate intelligence taught as measurable biology. PRI/PRM, the Reciprocal Tension Membrane / dura (falx cerebri, tentorium cerebelli, falx cerebelli, diaphragma sellae), and "dura as buttress" (W.G. Arbuckle, DO — cite historically) are cranial-osteopathic *models under investigation*. The dural anatomy is real; the PRM claims are the hypothesis.

Ch 4 — Anatomy for Airway Function ESTABLISHED

Viscerocranium & sphenoid; hyoid apparatus; mandibular plane. Solid, mainstream base.

Ch 5 — Tensegrity / Biotensegrity CORE FRAMING

Fuller/Snelson; **biotensegrity** — **Stephen Levin, MD**. The body as a pre-stressed, load-sharing network; **motion is distributed, deformation is shared — the system yields**. Suture micromotion is real; the sleep/airway significance is what CSA tests. CSA **applies** this — it did not invent it.

PART II

Sleep Science & Airway Mechanics

Course map: Week 1 continued

Ch 6 — The hidden epidemic (awareness)

Not only "overweight men who snore." Watch **children** (narrow palates, mouth breathing), **women** (often misread as insomnia/anxiety), **athletes** (forward-head from tech/training), **veterans** (neck/cranial trauma). DCs *see structure* — FHP, flattened lordosis, asymmetric nasal airflow, high/ retruded palate, pelvic torsion — mechanical predisposition visible before diagnosis.

Ch 7 — Snoring & the airway as a suspended tube

Snoring = turbulent flow through a narrowing airway. The airway is a flexible tube suspended by bone, muscle, fascia; when support or tone drops, negative pressure pulls the walls inward. Roots: FHP, retruded mandible/TMJ, cranial-base flexion, maxillary constriction, pelvic torsion. Nestor's *Breath* (with Stanford's Jayakar Nayak): mouth breathing narrows the airway — *form dictates function*.

"Correct the frame — quiet the sound."

Ch 8 — UAOSA, Central apnea & hypopnea

- **UAOSA** — mechanical collapse (effort continues). "UAOSA is the collapse of structure, not the failure of will."
- **Central apnea** — the brainstem **pre-Bötzinger complex** is the pacemaker; central events = no effort. (*The idea that cranial/dural mechanics influence the medullary environment is a hypothesis — never stated as fact.*)
- **Hypopnea** — partial collapse ($\sim \geq 30\%$ airflow drop). "A hypopnea is the biomechanical whisper before the body screams with apnea." The DC's early window — measured, co-managed.

Ch 9 — Testing & severity PHYSICIAN-READ

AHI (AASM): Normal <5 · Mild 5–14 · Moderate 15–29 · Severe ≥ 30 .

Test	Use
Home Sleep Test (HST)	DC-coordinated screening/telemedicine; confirms presence/severity
Polysomnography (PSG)	Gold standard; complex/central cases
Overnight pulse-oximetry	Pre-HST screen / progress tracking
Acoustic / airflow	Nasal resistance, flow limitation

Parameters: AHI · ODI · snore index · body-position correlation · HRV. **Classification:** UAOSA (airflow stops, effort continues) · Central (both stop) · Mixed. DC correlates numbers with structure **within scope**.

Ch 10 — The Consequence Standard in practice

Document **before/after:** posture, CBCT/lateral-ceph airway volume, HST metrics, patient-reported outcomes. **Report responders and non-responders.** Retest ~ 30 –60 days. Improvement shows structure *contributed in that case* — never a cure. Data → the **CSA Outcomes Registry**.

PART III

VAET™ as a Framework

Course map: Week 2 protocol + Week 3 biomechanics

Ch 11 — What VAET Is

Not a new technique — a **condition-specific application framework** sequencing established methods (CFR/BNS, SOT, AK, CBP, postural foundation) for the airway indication. A legitimate **early structural intervention, honestly framed** — measured, co-managed, **never** a replacement for CPAP, appliances, or surgery.

Ch 12 — The VAET lineage **NAMES CONFIRMED**

Airway-expansion cranial work has a real chiropractic lineage: **Bilateral Nasal Specific (BNS)** — founded by **Dr. J. Richard Stober, DC, ND**, an endonasal cranial adjusting procedure developed in the 1960s–1970s → **Dean Howell, ND** — NeuroCranial Restructuring (NCR) → **Adam Del Torto, DC** — Cranial Facial Release (CFR) → **Robert Zeravica, DC** — VAET (adds SOT/AK pelvic-cranial integration, targets the viscerocranium for sleep and airway). Contemporary collaborator: **Glenn Frieder, DC**. Honor the lineage; keep claims measured.

Ch 13 — CFR / BNS **WITH SAFETY**

Balloon mobilization of cranial sutures via the nasal passages, in a full-body model. **Safety:** sterile nasal balloons only; contraindications include recent nasal surgery, severe sinus infection, uncontrolled hypertension; informed consent; document nasal patency pre/post.

Ch 14 — SOT, CBP & postural foundation

SOT pelvic categories I–III (incl. the Lovett-Brother TMJ↔SIJ pairing); CBP cervical-lordosis correction — integrated and **measured** (pre/post angles). "Lordosis change → reduced AHI" is a *per-case hypothesis to test*.

Ch 15 — Cerebrospinal Fluid

CSF exchange is rhythmic and sleep-enhanced (glymphatic — established). Whether **cranial mechanics assist** it is *CSA's model under investigation*. State the clauses separately.

Ch 16 — Sequential, co-managed care model **THE RIGHT FRAMING**

Phase	Objective	Lead
Phase 1 — Mechanical restoration	Postural, cranial, TMJ, pelvic correction	DC
Phase 2 — Physiologic support	CPAP / oral appliance where indicated to stabilize oxygenation	MD / DDS
Phase 3 — Functional reintegration	Reassess; reinforce nasal breathing; taper devices only under the prescriber	DC-led team

Collaboration without collapsing scope — structure first, devices as needed, everyone in their lane.

PART IV

Whole-Person Support

Course map: Week 4 lifestyle · evidence-based only

Ch 17 — Lifestyle foundations

Sleep hygiene, nutrition, weight where relevant, **hydration**, movement, stress — evidence-based habit prescriptions, measured. A simple mnemonic (Air · Water · Salt · Oil / NEW START) can organize patient education using the plain, defensible versions.

Removed from the professional CE curriculum, and why. Energy/frequency "medicine" (scalar, Rife, ozone insufflation, EMF-foil/copper- grounding); **structured/alkaline "Kangen" water, microclustering, ORP, deuterium-depleted water**; the grander hydration claims ("water is the primary communicator of biological intelligence"); and **commercial product endorsements** (Enagic, etc.) are **out**. They read as pseudoscience to the MD/DDS/college audience, some carry safety/regulatory concerns, and endorsements create conflict-of-interest exposure. Plain hydration guidance stays; the special-water science does not.

PART V

Applied Clinical Protocols

Course map: Week 2 protocol

Ch 18 — Apnea Awareness Screening Script (3-5 min)

Opening: "Many sleep problems begin as structural or breathing issues that happen when we rest. I'll ask a few questions to understand how your airway and posture may be affecting your sleep."

Symptoms (1-5): snoring/gasping/choking? told you stop breathing? wake tired? morning headache/dry mouth/congestion? wake to change position from discomfort? (*flag → Insomnia-Positional Pain™*)

Breathing/behavior (6-10): mouth-breathing? both nostrils clear now? daytime sleepiness/caffeine? wake startled/short of breath? prior apnea/asthma/sinus dx?

Structural/postural (11-15): told you have FHP/neck misalignment? jaw clenching/TMJ/grinding? nasal surgery/orthodontics/extractions that changed the bite? shoulder rounding/upper-back tension? chronic pelvic/low-back imbalance?

Lifestyle (16-20): alcohol/nicotine/sedatives near bed? hours slept & how refreshed? exercise/stretch? anxiety/stress at night? prior HST/sleep study?

Close: "This helps me understand how your structure, breathing, and sleep connect. The next step is to measure your sleep at home, then correct the mechanics that may be causing your fatigue."

Ch 19 — The Apnea Exam (structural, 8 components)

Section	Assess	Why
Postural	Head carriage, shoulders, cervical curve, pelvic rotation	FHP reduces airway volume; pelvic torsion alters dural tension
Craniofacial	Facial symmetry, palate height, maxillary width, dental crowding, nasal bridge	Constriction / retruded mandible reduce airway space
Airway	Mallampati (education, not dx), tongue posture, bilateral nasal airflow, uvular vibration	Patency & vibration potential
TMJ / mandibular	Condyle motion, clicks, clenching, masseter/temporalis/pterygoid tone	TMJ dysfunction correlates with obstruction/bruxism
Cervical	Occiput-C1-C2 alignment, SCM/scalene/hyoid tone	Alters hyoid & tongue position
Respiratory	Paradoxical breathing, diaphragm motion, ribcage restriction	Reflects compromised PRM
Neuro screen	CN IX/X/XII, gag, tongue deviation, swallow/phonation	Hypoglossal/vagal control of airway tone
Sleep-position sim	Supine → observe snoring/narrowing	Helps the patient <i>see</i> the problem

Optional objective tools: digital posture/CBP scan, lateral cervical X-ray, acoustic rhinometry, HST.

"Apnea is visible in posture, palpable in the neck, measurable in motion."

Ch 20 — Clinical interpretation & workflow

Pattern	Likely category	Next step
Snoring + FHP + mouth-breathing	UAOSA	HST; cranial/cervical eval; structural plan
Fatigue + micro-arousals + positional pain	Insomnia-Positional Pain™	Postural/cranial assessment; SOT/AK
Gasping + no effort + occiput/atlas fixation	Central apnea	Neuromechanical eval; refer for sleep study

Workflow: Screen → order HST (physician-read) → interpret AHI/desat vs. structure → consult/educate → structural care plan (co-managed) → **retest 30-60 days**. Scope: screen, correlate, co-manage, refer. De-identified data → Outcomes Registry (responders + non-responders).

PART VI

Professional Development

Course map: Week 4 — Communication & Practice Integration

Ch 21 — The CSA continuing-education model

CE approved by the **California Board of Chiropractic Examiners** (state-specific). Competencies: screen; interpret HST in a biomechanical context within scope; design a co-managed plan; document outcomes.

Pathway: workbook → documented case → practical competence → exam → **Certified Chiropractic Sleep & Upper-Airway Specialist™**.

Ch 22 — Ethical growth & collaboration

Build a sleep/airway practice on **data and collaboration**, not disparagement. (*Cut from professional copy: device "dishonorable mentions" and slogans — they violate the no-disparagement rule and undercut co-management.*) Describe named mentors/influences accurately; imply no endorsement without consent.

Ch 23 — Outcomes, recognition & future

Publish de-identified outcomes; pursue academic/CE recognition on the strength of **data**; expand tracks as evidence supports. (Keep EDD/VA disability documentation as a separate line of work — not folded into the CE curriculum.)

Appendices and Editor's Checklist

Appendices: A. Foundation (faith, marked) · B. Mallampati chart (education) · C. Postural checklist · D. SOT pelvic-category flow chart · E. Apnea Exam form (8 components + asymmetric palatal-yield) · F. Screening Script (20 Qs + interpretation guide) · G. Pre/Post outcomes worksheet (responder + non-responder) · H. VAET lineage page (verify dates/citations) · I. References (AASM; craniofacial-OSA; Levin biotensegrity; glymphatic/CSF-in-sleep; Nestor *Breath*).

Editor's checklist before print

- Verify credentials/abbreviations **and lineage names/dates** against sources.
- State CE as "California Board of Chiropractic Examiners" (per-state).
- Confirm no energy/structured-water/product-endorsement content re-entered.
- No device disparagement. Devices = Phase 2.
- Every mechanism labeled established vs. hypothesis; no cure claims; no "instead of CPAP."
- Faith only in the bracketed Foundation section; secular twin leads clinically.
- One brand + domain: Chiropractic Sleep Academy™ / .net.
- Attorney review of scope ("diagnose" only within state scope) and CE/liability.